

		Plaintiffs to carefully review all procedural requirements in the future. In order to avoid further delay in this case, the court will grant leave for Defendant to file a demurrer and/or a motion to strike with respect to the Fourth Amended Complaint that use(s) the same reservation number(s) as Defendant's demurrer and motion to strike that are currently pending. This will allow Defendant's demurrer and/or a motion to strike with respect to the Fourth Amended Complaint to be heard on the date of the pending demurrer and motion to strike – May 20, 2024 – as long as Defendant's demurrer and/or a motion to strike with respect to the Fourth Amended Complaint are filed and served in compliance with the Civil Procedure Code. Plaintiffs shall give notice of this ruling.
10	Torres vs. Fountain Valley Regional Hospital and Medical Center 30-2023-01311157	<u>Demurrer</u> Defendant Fountain Valley Regional Hospital and Medical Center's Demurrer to the First Cause of Action for Elder Abuse Contained in Plaintiff's First Amended Complaint is OVERRULED. Defendant Fountain Valley Regional Hospital and Medical Center shall file an answer or other pleading in response to the First Amended Complaint within 10 days of service of the notice of ruling. (See Cal. Rules of Ct. rule 3.1320(j).) Defendant Fountain Valley Regional Hospital and Medical Center (Defendant Hospital) demurs to the 1st Cause of Action for Elder Abuse of the First Amended Complaint (FAC) filed by Plaintiff Maria Guadalupe Torres. <u>Standard for Demurrer</u> In ruling on a demurrer, a court must accept as true all allegations of fact contained in the complaint. (<i>Blank v. Kirwan</i> (1985) 39 Cal.3d 311, 318.) A demurrer challenges only the legal sufficiency of the affected pleading, not the truth of the factual allegations in the pleading or the pleader's ability to prove those allegations. (<i>Cundiff v. GTE Cal., Inc.</i> (2002) 101 Cal.App.4th 1395, 1404-05.)

		Questions of fact cannot be decided on demurrer. (<i>Berryman v. Merit Prop. Mgmt., Inc.</i> (2007) 152 Cal.App.4th 1544, 1556.) Because a demurrer tests only the sufficiency of the complaint, a court will not consider facts that have not been alleged in the complaint unless they may be reasonably inferred from the matters alleged or are proper subjects of judicial notice. (<i>Hall v. Great W. Bank</i> (1991) 231 Cal.App.3d 713, 718, fn.7.) Although courts should take a liberal view of inartfully drawn complaints, (see Code Civ. Proc., § 452), it remains essential that a complaint set forth the actionable facts relied upon with sufficient precision to inform the defendant of what plaintiff is complaining, and what remedies are being sought, (<i>Leek v. Cooper</i> (2011) 194 Cal.App.4th 399, 413). Bare conclusions of law devoid of any facts are insufficient to withstand demurrer. (<i>Schmid v. City and County of San Francisco</i> (2021) 60 Cal.App.5th 470, 481; see Code Civ. Proc., § 425.10, subd. (a).) <u>1st Cause of Action (Elder Abuse)</u> A cause of action brought under the Elder Abuse Act (Act), Welfare and Institutions Code §§ 15600, <i>et seq.</i>, allows the plaintiff to seek certain enhanced remedies when the plaintiff proves: 1) “by clear and convincing evidence” that 2) “defendant is liable for physical abuse as defined in Section 15610.63, neglect as defined in Section 15610.57, or abandonment as defined in Section 15610.05,” and that 3) the defendant committed the abuse with “recklessness, oppression, fraud, or malice.” (Welf. & Inst. Code, § 15657, subd. (a).) The rule that “statutory causes of action must be pleaded with particularity” applies to claims made under the Elder Abuse Act. (<i>Covenant Care, Inc. v. Superior Court</i> (2004) 32 Cal.4th 771, 790.) <u>Neglect</u> The Act defines “neglect” to mean “[t]he negligent failure of any person having the care or custody of an elder or a dependent adult to exercise that degree of care that a reasonable person in a like position would exercise.” (Welf. & Inst. Code, § 15610.57 subd. (a)(1); see also Welf. & Inst. Code, § 15610.27 [defining “elder” as “person residing in this state, 65 years of age or older”].)
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		Examples of neglect include, but are not limited to, the following: (1) Failure to assist in personal hygiene, or in the provision of food, clothing, or shelter. (2) Failure to provide medical care for physical and mental health needs. A person shall not be deemed neglected or abused for the sole reason that the person voluntarily relies on treatment by spiritual means through prayer alone in lieu of medical treatment. (3) Failure to protect from health and safety hazards. (4) Failure to prevent malnutrition or dehydration. (Welf. & Inst. Code § 15610.57, subd. (b).) <u>Care or Custody</u> As an initial matter, Section 15610.57(a)(1) requires that, for there to be neglect, the plaintiff must have been under the “care or custody” of the defendant. The Supreme Court has held that this requires plaintiff to prove “the existence of a robust caretaking or custodial relationship — that is, a relationship where a certain party has assumed a significant measure of responsibility for attending to one or more of an elder’s basic needs that an able-bodied and fully competent adult would ordinarily be capable of managing without assistance.” (<i>Winn v. Pioneer Medical Group, Inc.</i> (2016) 63 Cal.4th 148, 158.) This means that “the distinctive relationship contemplated by the Act entails more than casual or limited interactions.” (<i>Ibid.</i>) In determining whether such a relationship exists, “the focus . . . is on the nature and substance of the relationship between an individual and an elder or a dependent adult.” (<i>Ibid.</i>) The court noted that the “defendant’s professional standing or expertise” was not as important as whether “an elder or dependent adult depends on another for the provision of some or all of his or her
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		fundamental needs” or “the defendant[] had explicitly assumed responsibility for attend to the elders’ basic needs.” (<i>Id.</i> at pp. 158-160.) The FAC alleges that Defendants are liable for Elder Abuse because they neglected the Plaintiff, even though they were “care custodians” for the Plaintiff. (See, e.g., FAC, ¶¶ 12-13.) Defendants contend that this cause of action fails because the FAC does not sufficiently plead a robust caretaking or custodial relationship. Here, the FAC alleges that “[Defendants were to provide ‘care or services’ to dependent adults and the elderly, including [Plaintiff] and were to be the ‘care custodians’ of Plaintiff . . . and in a trust and fiduciary relationship with [Plaintiff] and upon whom [Plaintiff] depended in the manner and degree hereinafter alleged for the performance of their custodial duties.” (FAC, ¶ 12.) The FAC asserts that Plaintiff was under Defendant Hospital’s care for at least five days, during which there was a skin integrity care plan that required Defendant Hospital to monitor Plaintiff’s skin for redness and breakdowns at least twice a day; reposition Plaintiff’s at least every two hours; and report, notate, and document Plaintiff’s medical issues. (See FAC, ¶¶ 16-19.) The FAC also pleads that Defendant Hospital “knew that due to [Plaintiff’s] physical condition, she was unable to provide for her own basic needs and was dependent on them for meeting her basic needs such as nutrition, hydration, as well as medical care and health services, assistance and monitoring with feeding, the provision of safety and assistance devices to prevent accidents, and the implementation of interventions to prevent skin breakdown and infections.” (FAC, ¶ 29.) The FAC sufficiently pleads that Plaintiff was in the “care or custody” of Defendant Hospital, that Plaintiff depended on Defendant Hospital for the provision of some or all of her fundamental needs, and that Defendant Hospital had explicitly assumed responsibility for attend to Plaintiff’s basic needs. <u>Professional Negligence Not Sufficient</u> The Supreme Court has reviewed the purposes and legislative history of Section 15657 and
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		determined that “the statute was not meant to encompass every course of behavior that fits either legal or colloquial definitions of neglect.” (<i>Winn v. Pioneer Medical Group, Inc.</i> (2016) 63 Cal.4th 148, 160.) Instead, the Act seeks to guard against “certain situations [that] place elders and dependent adults at heightened risk of harm” and for which “heightened remedies relative to conventional tort remedies are appropriate as a consequence.” (<i>Id.</i> at pp. 160-161; see also <i>Delaney v. Baker</i> (1999) 20 Cal.4th 23, 36-37.) Therefore, “neglect [as a form of abuse under the Act] refers not to the substandard performance of medical services but, rather, to the ‘failure of those responsible for attending to the basic needs and comforts of elderly or dependent adults, regardless of their professional standing, to carry out their custodial obligations.’” (<i>Covenant Care, Inc. v. Superior Court, supra</i>, 32 Cal.4th at p. 783, quoting <i>Delaney v. Baker, supra</i>, 20 Cal.4th at p. 34.) “Thus, the statutory definition of ‘neglect’ speaks not of the <i>undertaking</i> of medical services, but of the failure to <i>provide</i> medical care.” (<i>Covenant Care, Inc. v. Superior Court, supra</i>, 32 Cal.4th at p. 783, italics original.) In addition, “[t]o recover the enhanced remedies under the Elder Abuse Act from a health care provider, a plaintiff must prove more than simple or even gross negligence in the provider’s care or custody of the elder.” (<i>Carter v. Prime Healthcare Paradise Valley</i> (2011) 198 Cal.App.4th 396, 405, citations omitted.) Instead, [The plaintiff] must show reckless, oppressive, fraudulent, or malicious conduct. The latter three categories involve “intentional,” “willful,” or “conscious” wrongdoing of a “despicable” or “injurious” nature. “Recklessness” refers to a subjective state of culpability greater than simple negligence, which has been described as a “deliberate disregard” of the “high degree of probability” that an injury will occur. (<i>Delaney v. Baker, supra</i>, 20 Cal.4th at p. 31, citations omitted.) As the Supreme Court has explained, “[i]n order to obtain the Act’s heightened remedies, a plaintiff must allege conduct essentially equivalent to
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		conduct that would support recovery of punitive damages." (<i>Covenant Care, Inc. v. Superior Court</i>, <i>supra</i>, 32 Cal.4th at p. 789.) Thus, "the enhanced remedies are available only for 'acts of egregious abuse against elder and dependent adults.'" (<i>Carter v. Prime Healthcare Paradise Valley</i> (2011) 198 Cal.App.4th 396, 405, quoting <i>Delaney v. Baker</i>, <i>supra</i>, 20 Cal.4th at p. 34.) <i>Carter v. Prime Healthcare Paradise Valley</i> (2011) 198 Cal.App.4th 396 applied these principles. In that case, the decedent was treated at both a hospital and a skilled nursing facility before he passed away. (See <i>id.</i> at pp. 401-402.) The Court of Appeal affirmed the trial court's ruling sustaining the hospital's demurrer because plaintiffs had not alleged any facts to show that the hospital had neglected the decedent. (<i>Id.</i> at pp. 407-407.) The Court of Appeal held that while plaintiffs had alleged that there had been a "failure to treat [decedent's] pressure ulcers, administer prescribed antibiotics or stock the crash cart; false documentation; purposefully inadequate testing for medications," plaintiffs had not plead facts sufficient to show that any of this conduct rose to the level of neglect. (<i>Id.</i> at pp. 408-410.) The Court of Appeal also ruled that standard professional negligence was not sufficient to amount to neglect. (<i>Ibid.</i>) Defendant Hospital contends that the Elder Abuse Cause of Action fails to allege neglect with particularity and that, at most, Plaintiff alleges professional negligence and not a total failure to provide the care required by Plaintiff. The FAC alleges that "on numerous days during [Plaintiff's] hospitalization at [Defendant Hospital], [Defendant Hospital's] staff failed to inspect [Plaintiff's] skin every shift and reposition her every two hours," which caused injury, pain, and suffering to Plaintiff. (FAC, ¶¶ 20-21.) The FAC also pleads that the failure of Defendant Hospital to abide by the skin integrity care plan was a result of "deliberate understaffing at [Defendant Hospital] by its managing agents." (FAC, ¶ 22.) <i>Sababin v. Superior Court</i> (2006) 144 Cal.App.4th 81 provides guidance here. In that case, a
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		dependent adult suffering from Huntington's disease was placed in a rehabilitation center and died two months later due to a staph infection caused by a sacral skin ulcer. (<i>Id.</i> at p. 84.) The center brought a motion for summary judgment and for summary adjudication, arguing that the decedent was turned on a regular basis and that the ulcers did not develop while she was at the center. (<i>Id.</i> at pp. 86-87.) The plaintiffs presented evidence in opposition showing that the decedent's care plan required her skin to be monitored on a daily basis for redness and breakdown and that, when she was transferred from the center to a hospital, decedent had "red, squishy buttocks, a bleeding toe, and a bruised and reddened foot," none of which had been documented by the rehabilitation center. (<i>Id.</i> at p. 87.) The trial court granted the motion on the ground that plaintiff's evidence, even if believed, showed nothing more than negligence. (<i>Ibid.</i>) The Court of Appeal reversed, finding that the center's failure to follow the care plan and failure to check the skin of a resident with Huntington's could constitute "neglect" under the Act. (<i>Id.</i> at p. 90.) The Court of Appeal reasoned that the "trier of fact could find that when a care facility's employees ignore a care plan and fail to check the skin condition of a resident with Huntington's Chorea, such conduct shows deliberate disregard of the high degree of probability that she will suffer injury." (<i>Ibid.</i>) The FAC in this case alleges that Plaintiff "was unable to provide for her own basic needs and was dependent on them for meeting her basic needs." (FAC, ¶ 29.) The FAC also pleads that Defendant Hospital and its staff failed to follow Plaintiff's skin integrity care plan. (See FAC, ¶¶ 16-20.) The FAC then alleges that, as a result of Defendant Hospital's conduct, including its failure to inspect Plaintiff's skin every shift and reposition her every two hours, Plaintiff suffered from untreated ulcers near the sacrum that caused injury, pain, and suffering. (FAC, ¶¶ 21-22.) As in <i>Sababin v. Superior Court</i>, this is sufficient for a trier of fact to conclude that Defendant Hospital's conduct amounted to deliberate
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		disregard of Plaintiff's basic needs and to the failure to provide medical care. Thus, the court will overrule the demurrer. Plaintiff shall give notice of this ruling.
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